

Letters to the Editor

The editor welcomes all letters whether they are short case reports, preliminary reports of research, discussion or comments on papers published in the journal. Authors should follow the same guidelines given for the preparation and submission of articles on the inside back cover of each issue.

Cocaine users in Amsterdam

SIR—In December 1989 I published extensive findings related to patterns and consequences of cocaine use in a group of 160 experienced cocaine consumers from non-deviant sub-cultures in Amsterdam (Cohen, 1989). One of the conclusions in my summary was that 'developing a cocaine policy in Amsterdam that aims at tolerating a similar non-criminalized distribution model of cocaine as we already have for cannabis, deserves serious consideration'.

This conclusion, drawn from my consideration of available drug policy arguments, epidemiological data, my own and other research data, drew fire from Edwards (1991). He doubts the findings I use to legitimate my suggestion for what he describes as a 'non-criminological' distribution model. He claims that the group I investigated is biased in relation to 'Amsterdam's 694 680 citizens' and that my claim to be researching in non-deviant circles 'loses its credibility'. My inferences, he continues, are based on 'flawed methodology' leading to a conclusion 'which by no stretch of logic can be properly derived from the findings'.

Professor Edwards is right to question the generalizability of data derived from users of illicit drugs. Often samples or cases are taken from clinics, prisons, 'hotlines' or other treatment situations. Findings are then incautiously generalized to all users of the same drug(s). I gave examples of this sort of work in my book.

As an alternative, I searched for experienced cocaine users but not from deviant or otherwise biased catchment areas. Accordingly, I did not start any of the snowballs with contacts who were junkies, full-time criminals or prostitutes; and nor did I recruit any of the 160 respondents in treatment

centres, prisons or brothels. Again and again I explained in the book that this is what I meant by 'non-deviant' subcultures.

What relation to 'Amsterdam's 694 680 citizens' does this group have? Edwards is correct to claim that the group does not represent the city population. Experienced cocaine use is a rare phenomenon in Amsterdam. In 1987, previous year prevalence of cocaine use in Amsterdam's population of 12 years and older was 1.7% and previous month prevalence a mere 0.6% (Sandwijk, Westerterp & Musterd, 1988). These very low figures were confirmed in the 1990 household survey in the city (Sandwijk *et al.*, 1991). We can infer from these data that the group of experienced cocaine users I interviewed must be atypical for the city's population as a whole.

But were they representative for cocaine users? To assess this I did not compare my group of 160 users with the general population, but with the 'previous year' cocaine users in the 1987 household survey. As I reported in detail, the group collected by our way of snowball sampling was quite like the household survey users on demographic and other socioeconomic variables. This then permitted me to generalize from the other characteristics of the group of 160. Edwards might have taken the opportunity to comment on this unique cross validation of a sample drawn from a hidden population.

In many respects the group of 160 experienced cocaine users are 'deviant' when compared to the rest of the Amsterdam population. To begin with, they are experienced cocaine users, they visit bars and cafes more often, they have used drugs to a greater extent and we have reason to assume that periods of heavy alcohol consumption are more prevalent. Yet I cannot see how this or any other

indication of relatively non-standard behaviour can invalidate the main findings of this lengthy study. In one of the largest snowball recruited studies of cocaine use ever conducted there is ample evidence of sustained ability to control cocaine use over a long period of time. In Amsterdam, where conditions of non-criminalized individual cocaine use and small distribution already exist, this evidence clearly supports the conclusion attacked by Professor Edwards. In a forthcoming publication I will show that follow-up research done among 64 of the original 160 experienced cocaine users strengthens this conclusion.

PETER COHEN

*Vakgroep Soliale Geografie,
Faculteit Ruimtelijke Wetenschappen,
Universiteit van Amsterdam, Amsterdam,
The Netherlands.*

References

- COHEN, P. (1989) *Cocaine Use in Amsterdam in Non Deviant Subcultures* (Amsterdam, Instituut voor Sociale Geografie).
EDWARDS, G. (1991) Review, *British Journal of Addiction*, 86, pp. 797–798.
SANDWIJK, J. P., WESTERTERP, I. & MUSTERD, S. (1988) *Het gebruik van Legale en Illegale Drugs in Amsterdam* (Amsterdam, Instituut voor Sociale Geografie).
SANDWIJK, J. P., COHEN, P. & MUSTERD, S. (1991) *Licit and Illicit Drugs in Amsterdam* (Amsterdam, Instituut voor Sociale Geografie).

Opium-tea and prevalence of HIV-1 infection among intravenous drug users in Vienna, Austria, 1986–1991

SIR—In his article, “Poppy tea drinking in East Anglia”, London *et al.*, (1990) highlighted the reappearance of this traditional practice in the United Kingdom. In Austria, the use of poppy has a long tradition. Poppy seeds can be bought by anyone and everyone in any supermarket or drug store anywhere in Austria without any restriction. Its cultivation as an alternative crop is subsidized by the government in Austria.

The use and dissemination of poppy-heads and poppy-straw are not under governmental control. At Austrian bakeries and confectionaries about 20 different sweet dishes with poppy-seeds as an ingredient are sold. In the Catholic Austria, it is forbidden to eat meat on Fridays, therefore, many dishes containing poppy-seeds are traditionally pre-

pared on this day. Dried poppy-heads are sold in florist shops to make dried bouquets or gold-coloured Christmas decorations.

In addition to heroin, ‘opium-tea’ is consumed by some Austrian opiate addicts exclusively, or when heroin is difficult to obtain. This opium-tea is extracted from dried poppy-heads. Depending on the country of origin, they contain a considerable amount of substances that can be found in raw opium.

Beginning in December 1985, a standardized method to monitor HIV-1 infection levels and trends among injecting drug users (IVDUs) has periodically been administered at the drug dependence outpatient ward of the Psychiatric University Clinic of Vienna (Loimer *et al.*, 1990). This has helped to develop an effective programme to reduce the further spread of HIV-1 among drug users. A climate in which there is no stigma attached to HIV-1 testing was created at our outpatient clinic, and this made this study possible. All IVDUs treated at the clinic in December 1985/January 1986, December 1986/January 1987, April 1988, February 1989, February 1990 and February 1991 were asked to be tested anonymously for HIV-1 antibodies by means of a screening test (ELISA) and a supplemental test (Westernblot). However, some patients refused to be tested. Adhering to the policy of anonymity, some patients were retested and reinterviewed. All the patients were clinically examined and a standardized questionnaire about their drug use history and their drug use behaviour was administered. In the first sample (1986) HIV-1 antibodies were found in 8.5% (7); in the second sample in 14.5% (23); in the third in 27.7% (36); in 1989 the HIV-1 prevalence was 29.7% (58); and in 1990 the HIV-1 prevalence was 26.9% (42) and in 1991 28.5% (35). Attention should be drawn to the fact that HIV-1 prevalence has increased year by year among females. This reflects the likely emergence of the AIDS epidemic as a predominantly heterosexual disease in the near future.

Between 1988 and 1989 HIV-1 seroprevalence increased dramatically. The frequency of HIV-1 seroprevalence among IVDUs remained nearly stable between 1988 and 1989 and showed even a mild decline in 1990.

In Vienna however, an explosive spread of HIV-1 infection among IVDUs was not observed. In fact, the frequency of HIV-1 penetration remained low compared with figures from the Austrian state of the Tyrol (Fuchs *et al.*, 1985) or New York (Drucker, 1986). This might be due to the high

This document is a scanned copy of a printed document. No warranty is given about the accuracy of the copy. Users should refer to the original published version of the material.